IRAQI MINISTRY OF HEALTH (MOH) & WHO IRAQ

National Clinical Protocol for Hypertension Management in Primary Health Care

Republic of Iraq - Ministry of Health & WHO Iraq - Clinical Guidelines for PHCCs

Official Reference: Iraqi MOH Public Health Directorate / NCD Department Clinical Protocol

Official Web Portal: https://moh.gov.iq

1. National Non-Communicable Diseases (NCD) Framework

* Implemented across all Iraqi Primary Health Care Centers (PHCCs) under the WHO PEN package.
* Universal Mandatory Screening: Routine blood pressure measurement for EVERY citizen >=18 years.
* Standardized measurement after 5 minutes seated rest, arm at heart level, recording average of 2 readings.

2. Clinical Diagnostic Criteria in Iraqi PHCCs

* Sustained clinic BP >=140 mmHg systolic and/or >=90 mmHg diastolic.
* Confirmed across 2 to 3 separate clinical visits within a 2-4 week window.
* Immediate single-visit diagnosis and urgent intervention if BP >=180/110 mmHg or if target organ damage ex

3. Essential Diagnostic Laboratory Package in Iraqi PHCCs

* General Urine Examination (GUE) for proteinuria and microscopic hematuria.
* Fasting Blood Sugar (FBS) and HbA1c.
* Blood Urea and Serum Creatinine (for eGFR calculation).
* Serum Electrolytes: Potassium and Sodium (baseline and follow-up).
* Fasting Serum Lipid Profile (Total Cholesterol, Triglycerides, HDL, LDL).
* Baseline 12-lead Electrocardiogram (ECG) in main sector health centers.

4. Iraqi Essential Medicines List (EML) First-Line Medications

* Calcium Channel Blockers (CCBs): Amlodipine 5-10 mg orally once daily.
* ACE Inhibitors: Enalapril 5-20 mg orally once daily, OR ARBs: Losartan 50-100 mg orally once daily.
* Thiazide Diuretics: Hydrochlorothiazide (HCTZ) 12.5-25 mg orally once daily.
* Dual Combination Therapy: Initiated at low doses (e.g. Amlodipine + Enalapril/Losartan) when BP >=160/100

5. Chronic Disease Clinic (NCD) Follow-Up Schedule in Iraq

* Patient registration in the official PHCC Chronic Disease Registry with dedicated follow-up card.
* Monthly clinic visits until blood pressure is controlled (<140/90 general, <130/80 for DM/CKD).
* Quarterly follow-up (every 3 months) for stabilized patients with monthly medication dispensing.
* Comprehensive annual evaluation: Serum creatinine, urinalysis, lipid profile, fundus exam, ECG.

6. Mandatory Specialist Referral Guidelines

* Hypertensive Emergency: BP >180/120 mmHg with acute symptoms -> Immediate referral via Iraqi EMS (Dial 122
* Suspected Secondary Hypertension: Patients under 30 years of age, or sudden severe refractory elevation.
* Resistant Hypertension: Uncontrolled BP despite full doses of 3 drug classes including a diuretic.
* Renal Complications: Rapidly rising serum creatinine or heavy proteinuria.
* Gestational Hypertension: Elevated BP during pregnancy to prevent pre-eclampsia/eclampsia.